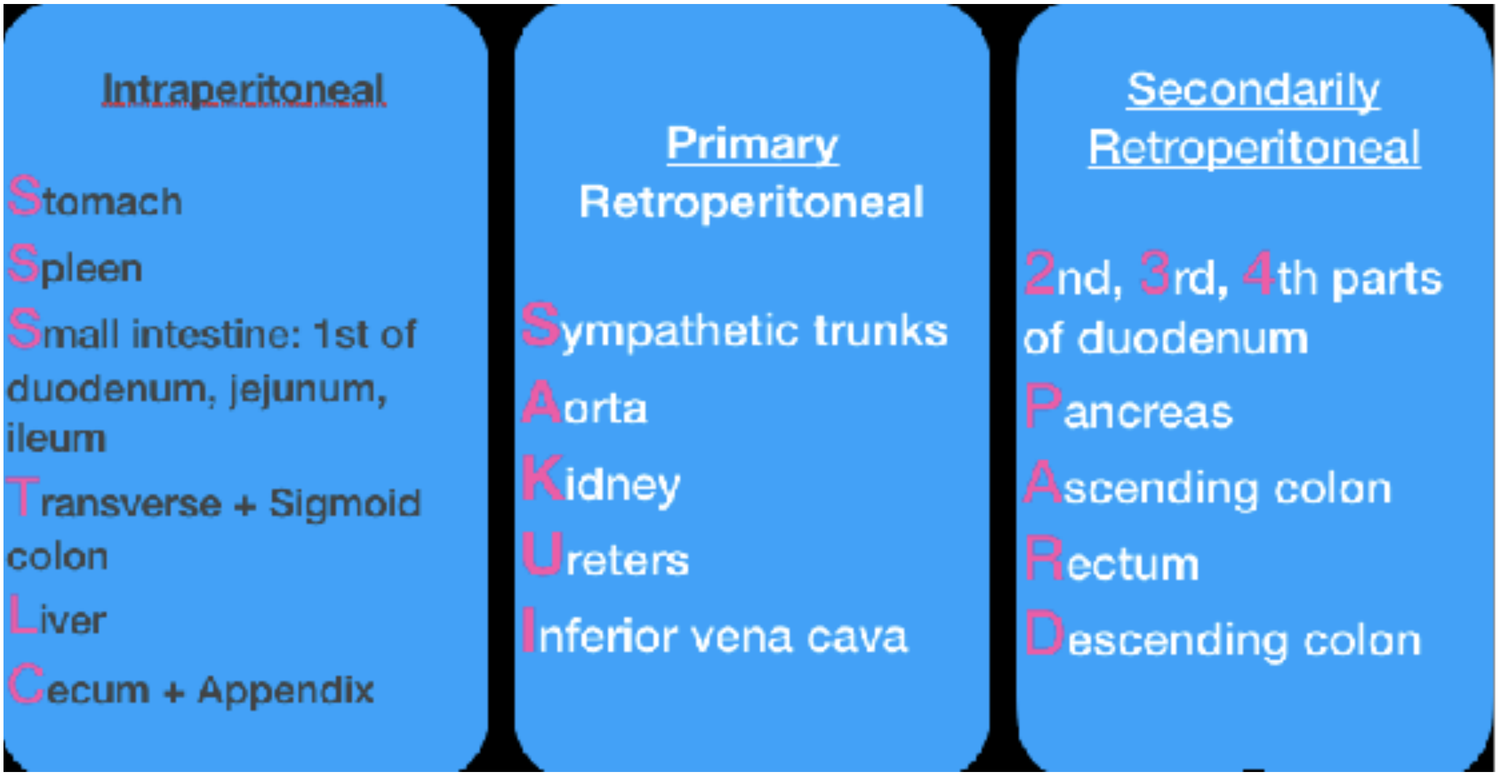


Intraperitoneal: Covered by visceral peritoneum, suspended by mesentery
1° Retroperitoneal: Developed retroperitoneally and stayed there. Fused to body wall
2° Retroperitoneal: Developed intraperitoneally, then moved retro. Immobile and fixed to body wall.

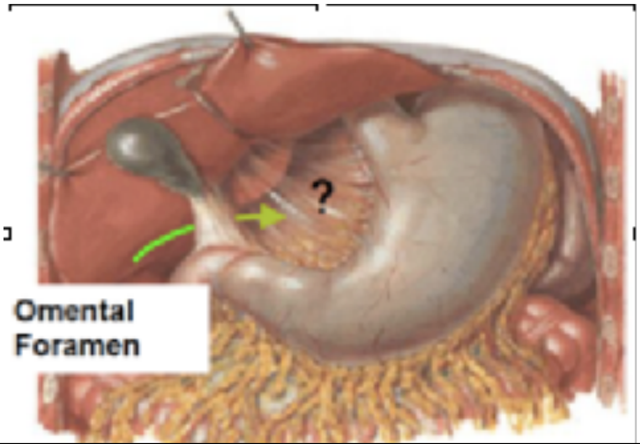
Ligaments
Double layer of peritoneum that stabilize structures

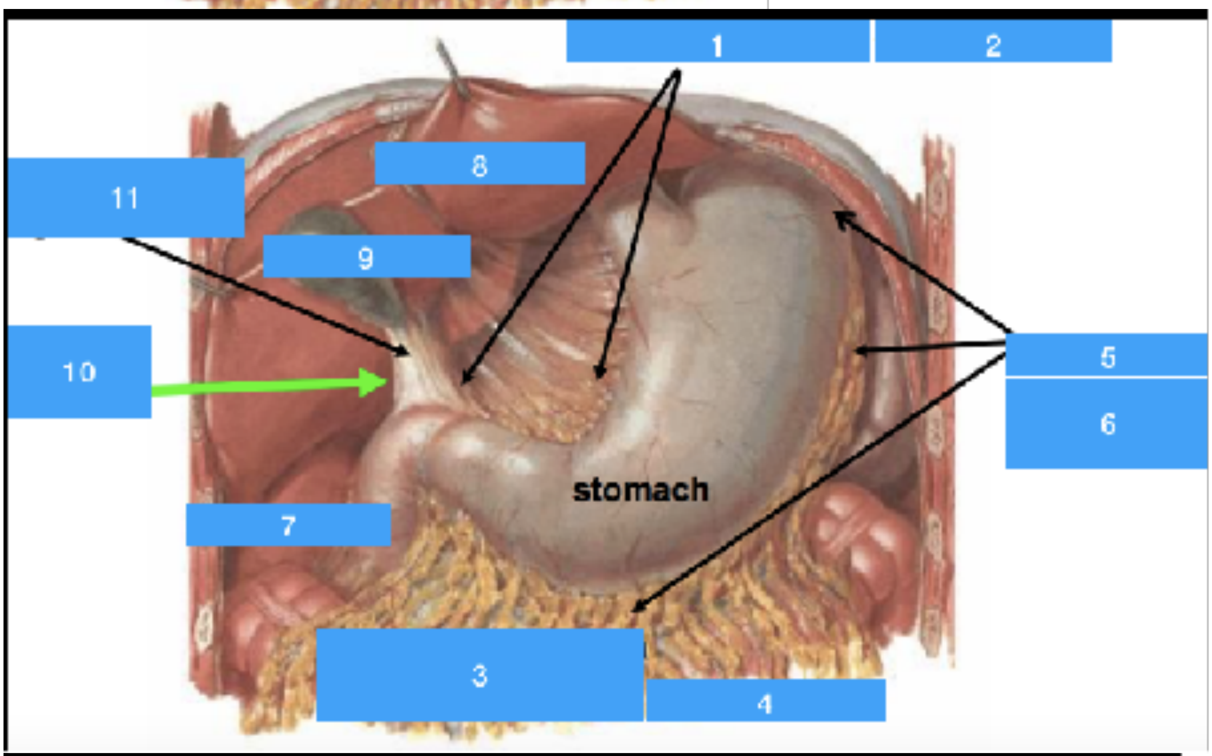
1. Omentum: Connect stomach to other organs
2. Greater Omentum
    1. Gastrophrenic, Gastrocolic, Gastrosplenic
3. Lesser Omentum: Stomach to Liver
    1. Hepatogastric (membranous): gastric arteries
    2. Hepatoduodenal (thick): Contains Portal Triad! (Portal vein, proper hepatic artery, common bile)

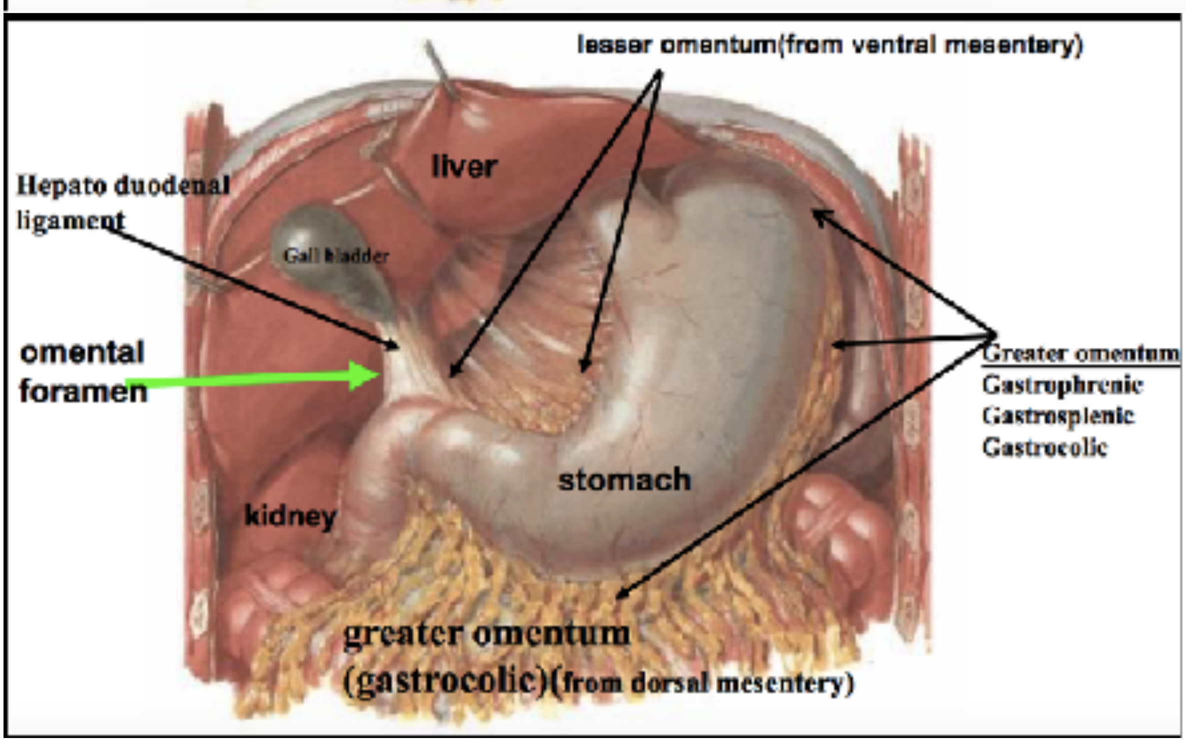
Omental Foramen (aka Epiploic, aka Winslow)

Boundaries of the omental foramen
which is the communication between the lesser sac and greater

Direction	Boundary
Anterior	Hepatoduodenal ligament with portal triad
Inferior	Duodenum (superior part)
Posterior	Inferior vena cava
Superior	Caudate lobe of the liver


?
Omental
Foramen


1
2
8
11
9
10
5
6
stomach
7
3
4


lesser omentum(from ventral mesentery)
liver
Hepato duodenal
ligament
Gall bladder
omental
foramen
Greater omentum
Gastrophrenic
Gastrosplenic
Gastrocolic
stomach
kidney
greater omentum
(gastrocolic)(from dorsal mesentery)

Flow of liquid

1. Omental/Eploic/Winslow Foramen: connect lesser/greater sac
2. Puncture stomach
    1. Posterior: Lesser sac
    2. Anterior: Greater sac
3. Lowest Points
    1. Laying down: hepatorenal recess (Morrison)
    2. standing (men) Retrovesicular Pouch
    3. standing (women): Vesico-Uterine Pouch (Douglas)

Paracolic Gutters

1. Connect infra/supracolic regions
2. Infection more likely through right gutter (left is narrower)